Respiratory Examination — OSCE Checklist

Deakin MD2 · CR7 · Time: 8 min (+2 min reading)

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Environment → General → Vitals → Hands → Arms → Face → Trachea → Nodes → Posterior chest → Anterior chest**. *Deakin order: examine the* **back first**, *then the front. Always compare side to side.*

1. Introduction

☐	Wash hands; introduce self; confirm patient ID
☐	Explain and gain consent: upper body clothing removed, only exposed when needed
☐	Offer a chaperone; privacy; warmth
☐	Position: sitting at 45° † (sit forward for the back)
☐	Equipment: stethoscope

2. Examination

	Step	What to do / say	Abnormal
Environment and general inspection			
☐	Environment	Oxygen (device, flow), inhalers, spacers, sputum pot, nebuliser †	Chronic lung disease
☐	Respiratory effort	Rate, accessory muscle use, tripod position, pursed lips †	Respiratory distress
☐	Speech	How much can they say in one breath?	Full sentences vs words
☐	Audible sounds	Cough, wheeze, stridor, voice change	Obstruction, recurrent laryngeal nerve
Vital signs			
☐	Full set	RR (count 30 s+), SpO_2 (on air?), HR, BP, temp	
Hands			
☐	Nails	Clubbing (look side-on, Schamroth's window †)	Lung cancer, bronchiectasis, ILD, CF
☐	Nicotine staining	Fingers and nails	Smoking
☐	Palms	Pallor	Anaemia
☐	Tremor	Fine tremor; flap if CO_2 retention †	Salbutamol use; hypercapnia
Arms			
☐	Skin	Bruising, thin skin	Long-term corticosteroids
Face			
☐	Eyes	Conjunctival pallor; Horner's syndrome (ptosis, miosis)	Anaemia; Pancoast tumour
☐	Sinuses	Facial tenderness	Sinusitis
☐	Mouth	Central cyanosis (under tongue)	Hypoxaemia
Neck			
☐	Trachea	Midline position (warn patient); tracheal tug	Deviation: pneumothorax, collapse, effusion
☐	Lymph nodes (from behind)	8 groups: submental, submandibular, anterior cervical, posterior cervical, pre-auricular, post-auricular, occipital, supraclavicular	Malignancy, infection
☐	JVP*	Raised JVP †	Cor pulmonale
Posterior chest (patient sitting forward, arms crossed †)			
☐	Inspect	Shape, symmetry, scars (thoracotomy, drains), muscle wasting, kyphoscoliosis	Previous surgery, restrictive disease
☐	Expansion	Hands on lower ribs, thumbs meeting in midline; deep breath — watch thumbs separate †	Reduced side = pathology
☐	Rib tenderness	Palpate ribs laterally	Fracture, costochondritis
☐	Percussion	Compare side to side, top to bottom, including axillae †	Dull: consolidation, collapse; stony dull: effusion; hyper-resonant: pneumothorax
☐	Breath sounds	Diaphragm, side to side, top to bottom; patient breathes through mouth	Vesicular vs bronchial; reduced; added sounds
☐	Added sounds	Crackles (fine/coarse), wheeze, pleural rub	Fibrosis, oedema, infection; airway narrowing
☐	Vocal resonance	"Say 99" while auscultating each area †	Increased = consolidation; reduced = effusion
Anterior chest (patient back at 45°)			
☐	Inspect	Look again for increased work of breathing, chest shape (barrel, pectus), scars	Hyperinflation (COPD)
☐	Palpate	Subcutaneous emphysema (crackling under fingers); apex beat †	Air leak; mediastinal shift
☐	Percussion	Include supraclavicular region and clavicles	As above
☐	Auscultation	Breath sounds, added sounds, vocal resonance including apices	As above
Legs*			
☐	Oedema, calves	Pitting oedema; calf swelling/tenderness †	Cor pulmonale; DVT (PE risk)

3. Closing and presenting

☐	Help patient dress; leave them as they arrived; wash hands
☐	If asked a question you can't answer, say you'll pass it to someone senior
☐	Completion: "I would review the observation chart, do a peak flow / spirometry, examine sputum, and request a chest X-ray †."
☐	Present: "Mrs X is comfortable at rest on room air, RR 14, SpO_2 98%. No clubbing or peripheral signs of respiratory disease. Trachea midline, expansion equal, percussion resonant throughout, vesicular breath sounds with no added sounds."

4. Quick reference †

Condition	Expansion	Trachea	Percussion	Breath sounds	Vocal resonance
Consolidation (pneumonia)	↓ affected side	Midline	Dull	Bronchial ± crackles	↑
Pleural effusion	↓ affected side	Away (if large)	Stony dull	↓ or absent	↓
Pneumothorax	↓ affected side	Away (tension)	Hyper-resonant	↓ or absent	↓
Collapse (lobar)	↓ affected side	Towards	Dull	↓	↓
COPD	↓ both, hyperinflated	Midline (tug)	Hyper-resonant	↓ ± wheeze	Normal / ↓
Pulmonary fibrosis	↓ both	Midline	Normal	Fine end-inspiratory crackles	Normal / ↑

- **Stridor** = upper airway (inspiratory); **wheeze** = lower airway (expiratory). Stridor is an emergency.
- Clubbing is **not** a feature of uncomplicated COPD — look for another cause (e.g. lung cancer).